NCCP 00237 · VERSION 6

Your treatment, explained

Advanced melanoma · NCCP 00237 v6 · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Dabrafenib alone — advanced melanoma

Reference treatment cycle

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · tafinlar-pi

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
Your treatment phases
Your 28-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Understanding your diagnosis and treatment goal
Your medicines and overall plan
Taking or receiving treatment safely
Tests, results and treatment decisions
What to expect between visits
Skin, body changes and daily care
Fertility, pregnancy and breastfeeding
Decisions, support and useful records
After a treatment hold, change or completion
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

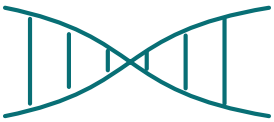
What each medicine does

To control advanced BRAF V600-mutated melanoma using dabrafenib alone, with regular review of benefit, fever and other side effects.

BRAF TARGETED TREATMENT

Dabrafenib

Tafinlar



Blocks an altered BRAF signal

Dabrafenib inhibits the altered BRAF V600 protein involved in melanoma growth. This guide describes dabrafenib alone for advanced melanoma. It does not include a second targeted medicine or a postoperative one-year treatment schedule.

Blocks an altered BRAF signal → Treatment plan and safety checks → Report new symptoms early

What to watch for

A temperature of 38°C or higher requires immediate contact and interruption under the current product information; follow the written plan and do not restart without team advice.

Sources: nccp · tafinlar-pi

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Fever / head

Dabrafenib

Fever, chills and headache can occur. At 38°C or higher, stop and call under the current product information; severe weakness, dizziness or confusion is urgent.

3 Skin

Dabrafenib

Rash, thickened skin and new skin cancers can occur. Report a changing lesion, a persistent sore or severe rash.

5 Kidneys / hydration

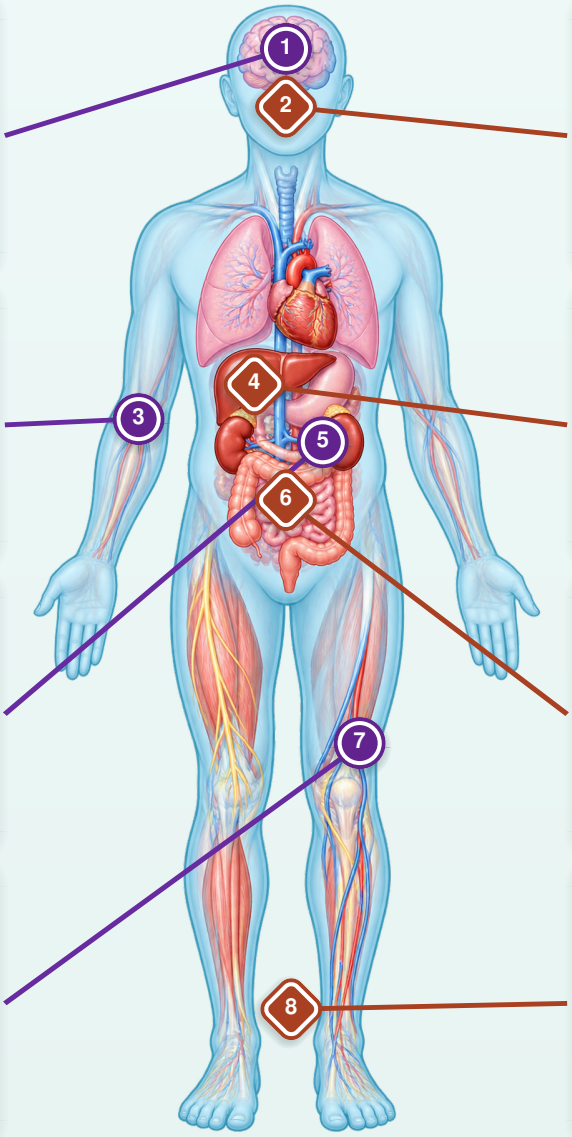
Dabrafenib

Fever, vomiting or diarrhoea can cause dehydration and kidney injury. Report reduced urine or inability to drink.

7 Joints / muscles

Dabrafenib

Joint pain, muscle pain or fatigue may affect activity. Report increasing pain or weakness and ask about suitable relief.



2 Eyes

Dabrafenib

Pain, redness, light sensitivity or blurred vision may be inflammation. Prompt eye assessment may be needed.

4 Liver

Dabrafenib

Routine blood tests look for liver changes. Yellow skin or eyes or dark urine need prompt review.

6 Abdomen / pancreas

Dabrafenib

Nausea and bowel changes can occur. Severe or persistent abdominal pain needs urgent assessment because pancreatic inflammation is possible.

8 Hands / feet

Dabrafenib

Sore thickened skin or redness on palms and soles can affect walking and grip. Report it early and ask for skin care tailored to your symptoms.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Your treatment phases

28-DAY REFERENCE CYCLE

Ongoing oral treatment

Ongoing oral treatment | Repeat only as prescribed

Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00237 v6: current regimen source · EMA Tafinlar product information, updated 19 May 2026

THE WHOLE TREATMENT JOURNEY

Dabrafenib alone — advanced melanoma: treatment and checks

Advanced melanoma

Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00237	Confirm BRAF V600 and check blood count, kidney and liver function, ECG, medicines and skin. Record existing skin lesions and relevant head-and-neck or other cancer assessments so later changes can be compared.	Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan.	If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.	Continue skin surveillance for up to six months after stopping, as specified by NCCP. Keep the agreed review for other new lesions and the individual response-imaging plan. No fixed tumour-marker schedule is assumed.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00237	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00237	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1

Ongoing oral treatment

28-day reference cycle

To control advanced BRAF V600-mutated melanoma using dabrafenib alone, with regular review of benefit, fever and other side effects.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00237

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

Treatment timing: displayed days 1–28

Treatment or check	Days in this window	What happens
Daily tablets / capsules	Days 1–28	Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.
Safety and symptom review	Days 1–28	Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan.

Schedule and review: Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

Holds, missed treatment and phase changes: Skip a missed dose if the next dose is due in less than six hours. Do not take two doses together to make up for it. If you vomit after a dose, do not take a replacement dose; take the next dose when due. The team must give instructions for restarting after fever or another treatment hold.

Why these bands stay broad: This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00237 v6: current regimen source ↗ · EMA Tafinlar product information, updated 19 May 2026 ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Complete timing and symptom notes

Day 1–28: At home each day

The usual starting dose is 150 mg twice daily, about 12 hours apart. Take capsules at least one hour before or at least two hours after food. Swallow whole with water. Use the prescribed strength and dose, and check the label again after any interruption or dose change. Skip a missed dose if the next dose is due in less than six hours. Do not take two doses together to make up for it. If you vomit after a dose, do not take a replacement dose; take the next dose when due. The team must give instructions for restarting after fever or another treatment hold.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

The usual starting dose is 150 mg twice daily, about 12 hours apart. Take capsules at least one hour before or at least two hours after food. Swallow whole with water. Use the prescribed strength and dose, and check the label again after any interruption or dose change.

STEP 2

Missed, vomited or held treatment

Skip a missed dose if the next dose is due in less than six hours. Do not take two doses together to make up for it. If you vomit after a dose, do not take a replacement dose; take the next dose when due. The team must give instructions for restarting after fever or another treatment hold.

STEP 3

Appointments and supply

Arrange blood tests, transport, medicine supply and interpreter or carer support. Check who reviews results and how you receive the decision before the next dose.

STEP 4

Paper copies and contacts

Keep the full handbook, three-page summary and wallet card together. The QR code is optional. Ask the team to replace every labelled example contact with your real oncology and out-of-hours details; ask for a new printed plan when treatment changes.

Sources: NCCP 00237 v6: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Supportive medicines and symptom plan

NCCP classifies dabrafenib as minimal-to-low emetogenic risk; anti-sickness treatment follows local and individual need. Agree a written fever plan and keep a thermometer and actual urgent-contact number. The team may prescribe fever treatment and hydration advice after assessment; fever medicine is not a substitute for contacting them. Anti-sickness treatment is selected individually.

2 Skin and daily care

Inspect skin and report new lumps, persistent warty growths, sores or changing moles. Report painful thickening or redness of palms and soles before it limits walking or hand use. Protect skin from the sun and attend monthly skin reviews.

3 Fertility, pregnancy and breastfeeding

Use effective contraception during dabrafenib alone and for two weeks after the last dose. Hormonal contraception may be less reliable; agree an effective alternative. A longer interval applies if another medicine requires it, so use your actual prescribed plan. Discuss fertility and breastfeeding before treatment.

Sources: nccp · tafinlar-pi

TREATMENT REFERENCE · 01

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Your diagnosis and stage

Keep the pathology report, original melanoma site, stage and known sites of spread. This guide concerns advanced disease that cannot be removed completely by surgery or has spread; it does not describe a postoperative recurrence-prevention course.

Sources: nccn-patient

The tumour test

A BRAF V600 test helps identify whether BRAF-targeted medicines are an option. A tumour mutation does not by itself mean an inherited family condition. Keep the exact result and ask how it changes your choices.

Sources: nccn-patient

Scans and local treatments

Record the scans used to follow known disease. Surgery or radiotherapy may have a separate role for a particular site; ask for their timing and risks to be added to the overall plan.

Sources: nccn-patient

New changes still matter

Report a new lump, changing mole, persistent skin sore or unexplained symptom. A response at one site does not establish that every other symptom is harmless.

Sources: nccn-patient

TREATMENT REFERENCE · 02

Your medicines and overall plan

To control advanced BRAF V600-mutated melanoma using dabrafenib alone, with regular review of benefit, fever and other side effects.

The schedule

Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

Sources: nccp · tafinlar-pi

How treatment works

Dabrafenib inhibits the altered BRAF V600 protein involved in melanoma growth. This guide describes dabrafenib alone for advanced melanoma. It does not include a second targeted medicine or a postoperative one-year treatment schedule.

Sources: nccp · tafinlar-pi

Important boundaries

This is dabrafenib monotherapy for advanced melanoma. Do not import the food, heart-testing, fertility or duration instructions of a different BRAF/MEK combination. The NCCP fever table states 38.5°C, while current EMA product information states interruption at 38°C for monotherapy; this guide uses the earlier current-product safety trigger and the team must reconcile the written plan.

Sources: nccp · tafinlar-pi

Evidence and choices

Median progression-free survival was 5.1 versus 2.7 months; hazard ratio 0.30, 95% CI 0.18–0.51. These are group results from that study, not a prediction for you. Advanced-disease monotherapy evidence, not postoperative dabrafenib/trametinib evidence. The comparator and treatment era do not establish superiority over current combinations.

Sources: break3 · nccp

TREATMENT REFERENCE · 03

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Administration

The usual starting dose is 150 mg twice daily, about 12 hours apart. Take capsules at least one hour before or at least two hours after food. Swallow whole with water. Use the prescribed strength and dose, and check the label again after any interruption or dose change.

Sources: nccp · tafinlar-pi

Missed or held treatment

Skip a missed dose if the next dose is due in less than six hours. Do not take two doses together to make up for it. If you vomit after a dose, do not take a replacement dose; take the next dose when due. The team must give instructions for restarting after fever or another treatment hold.

Sources: nccp · tafinlar-pi

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Sources: nccp · tafinlar-pi

Practical arrangements

Know where and when bloods are taken, how results reach the prescriber, and how to obtain the next supply or appointment. Ask for written instructions in a format you can use without a phone.

Sources: nccn-patient

TREATMENT REFERENCE · 04

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Confirm BRAF V600 and check blood count, kidney and liver function, ECG, medicines and skin. Record existing skin lesions and relevant head-and-neck or other cancer assessments so later changes can be compared.

Sources: nccp · tafinlar-pi

During treatment

Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan.

Sources: nccp · tafinlar-pi

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

Sources: nccp · tafinlar-pi

Response and follow-up

Continue skin surveillance for up to six months after stopping, as specified by NCCP. Keep the agreed review for other new lesions and the individual response-imaging plan. No fixed tumour-marker schedule is assumed.

Sources: nccp · tafinlar-pi

TREATMENT REFERENCE · 05

What to expect between visits

A quiet treatment day does not rule out later problems.

Act on new symptoms

If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.

Sources: nccp · tafinlar-pi

Supportive medicines

NCCP classifies dabrafenib as minimal-to-low emetogenic risk; anti-sickness treatment follows local and individual need. Agree a written fever plan and keep a thermometer and actual urgent-contact number. The team may prescribe fever treatment and hydration advice after assessment; fever medicine is not a substitute for contacting them. Anti-sickness treatment is selected individually.

Sources: nccp · tafinlar-pi

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

Sources: nccp · tafinlar-pi

When life is affected

Tell the team about fatigue, pain, appetite, sleep, mood and ability to work or care for others. Practical and supportive care can accompany anticancer treatment; you do not have to wait until treatment ends.

Sources: nccn-patient

TREATMENT REFERENCE · 06

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

Skin and sun

Inspect skin and report new lumps, persistent warty growths, sores or changing moles. Report painful thickening or redness of palms and soles before it limits walking or hand use. Protect skin from the sun and attend monthly skin reviews.

Sources: nccp · tafinlar-pi

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Sources: nccp · tafinlar-pi

Food, fluids and activity

Keep meals and fluids manageable and follow any individual fluid or diet restrictions. Gentle activity and rest can be adjusted to how you feel. Ask for dietetic or rehabilitation help if intake, weight, strength or mobility is falling.

Sources: nccn-patient

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Sources: nccp · tafinlar-pi

TREATMENT REFERENCE · 07

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Contraception and pregnancy

Use effective contraception during dabrafenib alone and for two weeks after the last dose. Hormonal contraception may be less reliable; agree an effective alternative. A longer interval applies if another medicine requires it, so use your actual prescribed plan. Discuss fertility and breastfeeding before treatment.

Sources: nccp · tafinlar-pi

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Sources: nccp · tafinlar-pi

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sources: nccp · tafinlar-pi

Sexual health

Tell the team about sexual symptoms, relationship concerns or practical difficulties with the agreed precautions. Ask for a private discussion and specialist support when needed.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Sources: break3 · nccp

Support alongside treatment

Ask about symptom control, psychological support, finances, transport, work and help at home. Palliative or supportive care can be used alongside anticancer treatment to improve comfort and function.

Sources: nccn-patient

A person you trust

If helpful, bring a family member or supporter and agree what information may be shared. Ask for an interpreter or accessible information when needed; use teach-back to check the schedule and urgent plan.

Sources: nccn-patient

Your document set

Keep consent information, the three-page summary, complete handbook, treatment card, test plan and real contact details together. The QR opens more information but essential instructions remain available in print.

Sources: nccn-patient

TREATMENT REFERENCE · 09

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

Continue skin surveillance for up to six months after stopping, as specified by NCCP. Keep the agreed review for other new lesions and the individual response-imaging plan. No fixed tumour-marker schedule is assumed.

Sources: nccp · tafinlar-pi

Restarting or changing treatment

Skip a missed dose if the next dose is due in less than six hours. Do not take two doses together to make up for it. If you vomit after a dose, do not take a replacement dose; take the next dose when due. The team must give instructions for restarting after fever or another treatment hold.

Sources: nccp · tafinlar-pi

A treatment summary

Record medicines and doses received, important side effects, admissions, last-dose date, ongoing symptom treatment and the clinician responsible for follow-up. Ask for an updated paper copy for your GP and other treating teams.

Sources: nccn-patient

Your next review

Confirm the next blood test, scan and review, who explains the result, and which symptoms should trigger contact sooner. Keep actual daytime and out-of-hours numbers accessible after treatment stops.

Sources: nccn-patient

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

Median progression-free survival was 5.1 versus 2.7 months; hazard ratio 0.30, 95% CI 0.18–0.51. These are group results from that study, not a prediction for you.

Sources: break3 · nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the treatment schedule?

Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

What if a dose is missed or held?

Skip a missed dose if the next dose is due in less than six hours. Do not take two doses together to make up for it. If you vomit after a dose, do not take a replacement dose; take the next dose when due. The team must give instructions for restarting after fever or another treatment hold.

Which symptoms need early contact?

If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.

Which tests are needed?

Confirm BRAF V600 and check blood count, kidney and liver function, ECG, medicines and skin. Record existing skin lesions and relevant head-and-neck or other cancer assessments so later changes can be compared. Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan. Continue skin surveillance for up to six months after stopping, as specified by NCCP. Keep the agreed review for other new lesions and the individual response-imaging plan. No fixed tumour-marker schedule is assumed.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

> **Try explaining the plan back:** Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00237 v6. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00237 v6: current regimen source
- EMA Tafinlar product information, updated 19 May 2026
- NCCN Guidelines for Patients: Melanoma, 2026 — supplied education reference
- Hauschild et al. BREAK-3, Lancet 2012

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-06.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6726/237_Dabrafenib.pdf

https://www.ema.europa.eu/en/documents/product-information/tafinlar-epar-product-information_en.pdf

https://www.nccn.org/patients/guidelines/content/PDF/melanoma-patient.pdf

https://pubmed.ncbi.nlm.nih.gov/22735384/